

Urethral Injury (After a Broken Pelvis)

Patient Information · Draining urine first, then repairing the urethra months later

WARWIKI

A broken pelvis can tear the **urethra**, the tube that carries urine out of the body. A tube through the lower belly drains urine first. Once the other injuries have healed, usually at **least 3 months later**, an operation removes the scar and **joins the healthy ends**. Most of these repairs last.

About This Injury

Car crashes, falls from a height and crush injuries can break the ring of bones that forms the pelvis. In men, the urethra can be stretched or torn just below the prostate, deep behind the scrotum. Signs include blood at the tip of the penis and not being able to urinate. After a complete tear, the gap almost always heals closed with scar, so a repair is needed later.

Benefits and Risks in Numbers

about
9 in 10

repairs worked without another procedure in large studies

about
1 in 3

men have erection problems from the injury itself. The repair adds about 3 in 100 more

fewer than
1 in 10

men leak urine after a successful repair

Numbers are averages from published studies. Some studies report more erection problems. Your own risk may differ; ask your team.

The First Step: A Suprapubic Tube

A soft tube is placed into the bladder through the lower belly, above the pubic bone (a **suprapubic tube**). It drains urine while you heal. It has its own handout. In some hospitals, a catheter is guided across the tear in the first days instead, if you are stable. Ask which plan you had.

Why Wait for the Repair?

Waiting lets blood and swelling in the pelvis clear, the scar settle and the bones heal, so you can lie safely in the position the operation needs. Repair in the first 2 days after injury causes heavy bleeding and worse results. Some surgeons repair selected men sooner, at 3 to 6 weeks.

Learn the Terms

Urethra

The tube that carries urine out of the body.

Pelvic fracture

A break in the ring of bones at the base of the spine and hips.

Suprapubic tube

A tube that drains the bladder through the lower belly.

Urethroplasty

Surgery to repair the urethra. Here the scar is removed and the ends are joined.

Urethrogram

An X-ray of the urethra with dye.

Cystoscopy

A look inside with a thin camera.

Bladder neck

The valve where the bladder meets the urethra. It helps keep you dry.

Women Can Be Injured Too

This injury is rarer in women and is often linked to a tear in the vagina. It is easy to miss, so tell your team about bleeding from the vagina or trouble urinating. In women, the tear is usually repaired early, within days, rather than months later.

Will the repair affect erections or bladder control? Erection problems after this injury usually come from the injury itself, not the repair. Treatments can help; ask your team. Leaking after the repair is uncommon. Your team checks the bladder neck beforehand, because it helps keep you dry. If leaking lasts, there are treatments, such as an artificial urinary sphincter.

Right after the injury	Next months	3 months or later	After the repair
A suprapubic tube drains urine.	Other injuries heal. You live with the tube.	X-rays and a camera check, then the repair.	Catheter about 2–3 weeks. Checks for at least a year.

Tests Before Surgery

- A **combined X-ray**: dye fills the bladder through the tube and the urethra from the tip at the same time, while you try to urinate. It shows how long the gap is.
- A **thin camera** passed through the tube looks at the bladder neck. Some men also have an **MRI**.
- A **urine test**. Any infection is treated before surgery.

What Happens During Surgery

- 1 You get general or spinal anesthesia and antibiotics.
- 2 A cut is made between the scrotum and the anus.
- 3 The scar is removed, and the healthy ends of the urethra are joined. If more room is needed, the surgeon may separate the erection chambers or remove a small wedge of the pubic bone. Rarely, a cut on the belly is also needed.
- 4 A catheter is placed through the penis to protect the join while it heals.

After Surgery

- The catheter usually stays about 2–3 weeks. An X-ray with dye checks the join before it comes out. Your team decides when the suprapubic tube comes out.
- If the narrowing comes back, it is usually in the first months. It can usually be treated again.
- Expect flow tests and check-ups for at least a year.

Call the office that did your surgery if you have:

- A temperature of **100.4 °F (38 °C)** or higher, or chills
- Redness, swelling or pain that is getting worse, or fluid leaking from the cut
- A catheter or tube that falls out. Do not put it back in yourself.
- A stream that is slowing after the catheter is out

Go to an emergency department if:

- No urine drains from the catheter or tube, or you cannot urinate, and your bladder feels full or painful
- Heavy bleeding, or urine like red wine or with clots
- You cannot reach the office and are worried

Three Things to Remember

1. A suprapubic tube drains urine first. The repair is usually done at least 3 months later.
2. About 9 in 10 repairs work without another procedure.
3. Erection problems usually come from the injury itself. Ask about treatment.